



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : هناء توفيق خطاب محمد
المحافظة : القاهرة الحي / المدينة : المطرية
رقم البطاقة : 27705170102067 رقم التليفون : 01112840576
العمر (تاريخ الميلاد) : 17-05-1977 المهنة : ربه منزل
العنوان : عزبه حماده
تاريخ تسجيل الإستمارة : 22-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : أورام ليمفاوية
نتائج الأبحاث : (مرفق)

التشخيص النهائي : أورام ليمفاوية
خطة العلاج : أبحاث أورام رنين مغناطيسي علي المخ والحجج بالصبغه
[11112]
قسم العلاج :
الأورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / د. / د. / د. / د. / د. / د. / د. / د. / د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: هناء توفيق خطاب محمد National ID: 27705170102067	
---	---

تقرير طبي

السيد
تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد أنه
--

A patient of lymphoma for Contrast-enhanced MRI of the brain and orbits

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحليل الشخصية



هناء
توفيق خطاب محمد

١٦ ش الجامع - طه قنديل - عزبة حماده
المطرية - القاهرة

٢٧٧٠٥١٧٠١٠٢٠٦٧

LZ0471251



٢٠٢٦/٠٩

٢٧٧.٥١٧.١٠٢.٦٧

بدون عمل

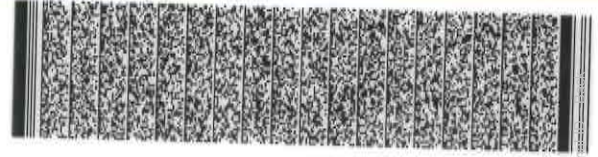
أنثى

مستلمة

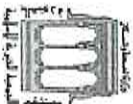
أنثى



البطاقة سارية حتى ٢٠٢٣/٠٩/١٦



[DOCTYPE:ID_BACK]



Patient Name : هاء توفيق خطاب
Patient No. : 0330138
Path No. :
Sex : Female
Age : 49 Y
Refereing Doc :

Request No. : 142776
Request Date : 09/07/2026
Verification Date : 27/07/2026
Financial Category : نقى - العجوزة
Department : Out Patient

Pathology Report

Path no: 364-26

Clinical data & Nature of specimen:

Right retroperitoneal mass, US guided biopsy.

Gross:

Multiple Fragmented greyish white tissue cores collectively measures 1.8x1.8 cm, totally submitted.

Microscopic:

Examination revealed cores of mass showed alternating hypo and hypercellularity formed of bland looking spindle cells. No evidence of significant atypia, mitosis or necrosis in sections examined.

Diagnosis:

Right retroperitoneal mass, US guided biopsy:

- Spindle cell neoplasm.
- Case is for mandatory immunophenotyping to start with (S100,SMA, Desmin, Ki67).
- Case is clinical and radiological correlation.

Signature

Dr/ Marwa Mahmoud Abd Elkader
FEBP.IFCAP





Patient Name: **هانى توفيق خليل محمد**
MRN: 10110226008792
D.O.B: 17/05/1977
IC No.: 27705170102067

Location: External Clinic
Sex: Female

Surgical Pathology-Diagnosis

Biopsy

Specimen Id 2606131086
Accession No P-26-7012
Sample Type TISSUE

Collection Date 13/06/2026 10:29 AM
Received Date 13/06/2026 10:33 AM
Doctor

CLINICAL HISTORY

A 49 years old female patient presented with inoperable retroperitoneal mass and retroperitoneal lymphadenopathy. Slide revision of incisional biopsy from mass and lymph nodes.

GROSS DESCRIPTION

Three referred paraffin blocks, and Three referred Hx & E stained slides labeled by patient's name in Arabic and coded 1059157, additionally designated 1, 2 and C.B

MICROSCOPIC DESCRIPTION

Examination of the received slides revealed:

- 1- Sections of lymph nodes showing preserved nodal architecture with Reactive Follicular Hyperplasia and sinus histiocytosis. No evidence of malignancy
- 2- Tissue fragment with fibrosis and collagenous stroma, entrapping blood vessels surrounded by lymph plasma cell aggregates. No evidence of malignancy
- 3- The slide received designated (C.B) show fragmented cores formed of spindled cells, fibroblasts and aggregates of large polyhedral cells with granular foamy cytoplasm, ? foamy/lipid laden histiocytes. Occasional hemosiderin laden macrophages are noted with mixed inflammatory cells.

IMMUNOHISTOLOGY

Marker study done revealed positive reaction to S100 and SOX10, while negative for CD68.

DIAGNOSIS

Retroperitoneal mass, Slide revision of core biopsy and mass content: Scanty, Low grade spindle cell neoplasm of neuronal origin. Exact nature and behavior of the tumor cannot be assessed on such scanty biopsy. Correlation with radiologic findings and larger biopsy are advised.

Category

A definitive diagnosis could not be established

Assistant Pathologist:
Dr. Reem Khaled

Registrar:

Signed by:
Dr. Eman Naguib, MD (Assistant Professor)

Reported and electronically endorsed by Dr. Eman Naguib Elhelaly

Dr.

Elia Anis Ishak

M. Sc. Ph. D. Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.

Shady Elia Anis

M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : هناءة توفيق خطاب

Analysis ID :  1059157

Age : 49 Y

Gender : Female

Conf. Code : 5603

Referred By Prof. Dr. : محمد عبد الوالد

Clinical Diagnosis : Inoperable retroperitoneal mass, retroperitoneal lymph node.

Nature of Specimen : Referred blocks and referred slides (retroperitoneal lymph node, mass wall incisional Biopsies and mass content cytology)

Receiving Date : 2026-05-19

Patient ID : 775419

PATHOLOGY REPORT

Gross :

Three blocks and three slides labeled as 26013062.

Microscopic :

Sections prepared from block (1) and examination of slide (1) received revealed multiple lymph nodes with preserved architecture, patent subcapsular sinus with many variably sized reactive follicles. The paracortex showed lymphoplasmacytic infiltrate.

No evidence of specific granuloma.

No evidence of malignancy.

Sections prepared from block (2) and examination of slide (2) received revealed fibrocollagenous tissue with moderate inflammatory reaction.

- Sections prepared from cell block(3) and examination of slide (3) received revealed cores of a tumor formed of bland spindle cells with oval and tapered nuclei in a fibrillary background showing intervening aggregates of xanthoma cells. No malignancy.

Diagnosis :

Inoperable retroperitoneal mass:

1- Retroperitoneal lymph node, Referred block and referred slide (Biopsy), **REACTIVE LYMPH NODES, CHRONIC LYMPHADENITIS.**

2- Mass wall, Referred block and referred slide (Incisional biopsy), **FIBROCOLLAGENOUS TISSUE WITH MODERATE INFLAMMATORY REACTION.**

3- Mass content, Referred cell block and referred slide(Cytology), **SPINDLE CELL NEOPLASM CONSIDER SCHWANNOMA.**

- CASE FOR IMMUNOPHENOTYPING FOR CONFIRMATION.

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak

Shady Elia

Elia anis

This report is electronically signed , any abrasion or modification would render it void .



عصارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع

تليفون : ٠١٢١٢٩٤٥١١٥ - ٠١٢٨٧٠٠٥٢١٧



میرزا محمد علی خان

Female 49 Y 2 M 11 D
30/4/2026 12:46:57 PM
11/5/2026 06:21:11 PM
13/5/2026 11:52:37 AM

Patient ID

Branch

91357

الفرع الرئيسي الثاني

Proton Biopsy And Mass Content

Pathology Report

grey, white and firm tissue fragments collectively measured 2.5 cm. The fragment was received in a container of 100 ml of 10% formalin.

...received in a syringe for cytological examination. Cell block was prepared.

sis

slides enclosed: 3

:Pathologist
Saeed, MD



١ - الحجر الأول

[illegible]

دقيق : ١١١١٥٧٤٧٠ - ت : ٧٣ - ١١٢٢٣٥٣ شارع التحرير

1059157

1059157
2005

جمهورية مصر العربية
بطاقة تحليل الشخصية



هنا
توفيق خطاب محمد

١٦ ش الجامع - طه قنديل - عزبة حماده
المطرية - القاهرة

٢٧٧.٥١٧.١.٢٠.٦٧

LZ0471251



٢٠٢٦/٠٩ ٢٧٧.٥١٧.١٠٢.٦٧

بدون عمل

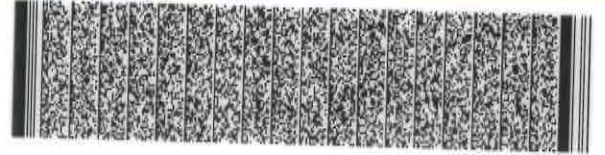
أنثى

مستلمة

أنثى



البطاقة سارية حتى ٢٠٢٣/٠٩/١٦



د/ فاطمة سيد عبد الباقي

دكتوراه وإستشاري علاج الأورام - جامعة عين شمس
زميل الكلية الملكية لعلاج الأورام - إنجلترا
عضو الجمعية الأوروبية لعلاج الأورام
مستشفى المطرية التعليمي
مستشفى في القاهرة التخصصي
مجموعة مستشفيات كليوباترا

Name: / د/ فاطمة سيد عبد الباقي

Date: 8/9/2026

* 50 yrs, Single, +ve FH د/ فاطمة سيد عبد الباقي
* myomectomy Post menopausal
of fibroid

* 3/2026: post renal obstructive uropathy
• bilat. double-J → ↓↓ S. creat 9

* 4/2026: MRI PA: Rt adnexal & lumbar
heterogenous mass

C.T chest: free

→ Surgical Exploration: Inoperable retroperitoneal
mass → multiple LN biopsy + mass biopsy
+ cytology

البيظة → Reactive lymphadenopathy + fibrocollagenous
tissue fragments + -ve cytology

البيظة → Revision of Pathology: mass → spindle cell
neoplasm schwannoma

⇒ IHC results

→ S100 & Sox10: +ve

CD68: -ve

⇒ low grade spindle cell
neoplasm of neuronal
origin

⇒ U/S guided biopsy: spindle cell
neoplasm for
IHC

منصة جوفتاف (فرم سابقاً)
المستشفى

اسم المريض ثلاثي:
رقم الملف:

تقرير طبي

السيد/

تحية طيبة وبعد،،

إيماء الطالب المقدم من بشأن تقرير طبي للمريض:
بقسم

نشرف بأن نوضح فيما يلي تقريراً عن حالته :
تاريخ وساعة الدخول

تاريخ وساعة الخروج

وبعد الكشف والفحوصات والمتابعة وجد انه:

١١١ / السيد العيصوي بلقيس حسن
حكيمه ستلا وفلاير ولسندر

تحريراً في

الوقت: التاريخ: / /

الطبيب المعالج

يتمتع ،،

اسم المريض ثلاثي:
رقم الملف:

تقرير طبي

السيد /

تحية طيبة وبعد،،،

إيماء للطلب المقدم من

قسم: رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته:

تاريخ وساعة الدخول تاريخ وساعة الخروج:

وبعد الكشف والفحوصات والمتابعة وجد انه:

Stable
NF1

والتي دخلت تحت ملاحظة العيون والجلد
والأشعة

تحريراً في

الوقت: التاريخ: / /

الطبيب المعالج

يختم ،،

المصدر /

إيماء الطالب المقدم من بشأن تقرير طبي للمريض..... أهبناك جو خصب
رقم طبي
يقسم.....

تاريخ وساعة الدخول تاريخ وساعة الخروج

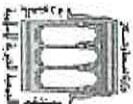
my best friend
Laila

25 June

2/13/20

الطبيب المعالج

يحيى



Patient Name : هناء توفيق خطاب
Patient No. : 0330138
Path No. :
Sex : Female
Age : 49 Y
Referring Doc :

Request No. : 142776
Request Date : 09/07/2026
Verification Date : 27/07/2026
Financial Category : نقى - العجوزة
Department : Out Patient

Pathology Report

Path no: 364-26

Clinical data & Nature of specimen:

Right retroperitoneal mass, US guided biopsy.

Gross:

Multiple Fragmented greyish white tissue cores collectively measures 1.8x1.8 cm, totally submitted.

Microscopic:

Examination revealed cores of mass showed alternating hypo and hypercellularity formed of bland looking spindle cells. No evidence of significant atypia, mitosis or necrosis in sections examined.

Diagnosis:

Right retroperitoneal mass, US guided biopsy:

- Spindle cell neoplasm.
- Case is for mandatory immunophenotyping to start with (S100,SMA, Desmin, Ki67).
- Case is clinical and radiological correlation.

Signature

Dr/ Marwa Mahmoud Abd Elkader
FEBP.IFCAP





Patient Name: **هانى توفيق عطلي محمد**
MRN: 10110226008792
D.O.B: 17/05/1977
IC No.: 27705170102067

Location: External Clinic
Sex: Female

Surgical Pathology-Diagnosis

Biopsy

Specimen Id 2606131086
Accession No P-26-7012
Sample Type TISSUE

Collection Date 13/06/2026 10:29 AM
Received Date 13/06/2026 10:33 AM
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IMMUNOHISTOLOGY

Marker study done revealed positive reaction to S100 and SOX10, while negative for CD68.

DIAGNOSIS

Retroperitoneal mass, Slide revision of core biopsy and mass content: Scanty, Low grade spindle cell neoplasm of neuronal origin. Exact nature and behavior of the tumor cannot be assessed on such scanty biopsy. Correlation with radiologic findings and larger biopsy are advised.

Category

A definitive diagnosis could not be established

Assistant Pathologist:
Dr. Reem Khaled

Registrar:

Signed by:

Dr. Eman Naguib, MD (Assistant Professor)

Reported and electronically endorsed by Dr. Eman Naguib Elhelaly

Dr.

Elia Anis Ishak

M. Sc. Ph. D. Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.

Shady Elia Anis

M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : هناءة توفيق خطاب

Analysis ID : 1059157

Age : 49 Y

Gender : Female

Conf. Code : 5603

Referred By Prof. Dr. : محمد عبد الوالد

Clinical Diagnosis : Inoperable retroperitoneal mass, retroperitoneal lymph node.

Nature of Specimen : Referred blocks and referred slides (retroperitoneal lymph node, mass wall incisional Biopsies and mass content cytology)

Receiving Date : 2026-05-19

Patient ID : 775419

PATHOLOGY REPORT

Gross :

Three blocks and three slides labeled as 26013062.

Microscopic :

Sections prepared from block (1) and examination of slide (1) received revealed multiple lymph nodes with preserved architecture, patent subcapsular sinus with many variably sized reactive follicles. The paracortex showed lymphoplasmacytic infiltrate.

No evidence of specific granuloma.

No evidence of malignancy.

Sections prepared from block (2) and examination of slide (2) received revealed fibrocollagenous tissue with moderate inflammatory reaction.

- Sections prepared from cell block(3) and examination of slide (3) received revealed cores of a tumor formed of bland spindle cells with oval and tapered nuclei in a fibrillary background showing intervening aggregates of xanthoma cells. No malignancy.

Diagnosis :

Inoperable retroperitoneal mass:

1- Retroperitoneal lymph node, Referred block and referred slide (Biopsy), REACTIVE LYMPH NODES, CHRONIC LYMPHADENITIS.

2- Mass wall, Referred block and referred slide (Incisional biopsy), FIBROCOLLAGENOUS TISSUE WITH MODERATE INFLAMMATORY REACTION.

3- Mass content, Referred cell block and referred slide(Cytology), SPINDLE CELL NEOPLASM CONSIDER SCHWANNOMA.

- CASE FOR IMMUNOPHENOTYPING FOR CONFIRMATION.

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak

Shady Elia

Elia Anis

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عصارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع

تليفون : ٠١٢١٢٩٤٥١١٥ - ٠١٢٨٧٠٠٥٢١٧



सं. ३०३

Code Number
Patient ID
Referred By Dr.
Branch

91357

الفرع الرئيسي الدقي

Diagnosis: Inoperable Retroperitoneal Mass
if the specimen: Retro-peritoneal Lymph No

Pathology Report

Designated specimens were received as:
 1. le grey reddish and yellowish

was received in a syringe for cytological examination. Cell block was prepared.

Copic Picture

showed five lymph nodes with reactive follicular hyperplasia. No metastatic malignancy (0/5).
showed fibro collagenous tissue with no epithelial lining and contained focal lymphocytic infiltrate.
showed foamy macrophages mixed with lymphocytes.

sis

Retropertoneal Mass, Lymph Node Biopsy, Mass Wall Incisional Biopsy And Mass Content Cytology: Correlate With Clinical And Radiological Findings.

slides enclosed: 3

ed by

Pathologist
Saeed, MD



ي - الحور الأول

حقوقي: - شارع التحرير أمام مقر البحوث - الصفحة ٧٣، ٥٤، ٢٠١١



1059157
2025

جمهورية مصر العربية
بطاقة تحليل الشخصية



هنا
توفيق خطاب محمد

١٦ ش الجامع - طه قنديل - عزبة حماده
المطرية - القاهرة

٢٧٧.٥١٧.٠١.٢٠.٦٧

LZ0471251



٢٠٢٦/٠٩ ٢٧٧.٥١٧.١٠٢.٦٧

بدون عمل

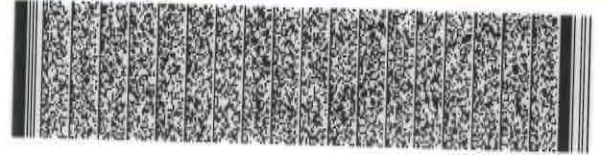
أنثى

مستلمة

أنثى



البطاقة سارية حتى ٢٠٢٣/٠٩/١٦



د/ فاطمة سيد عبد الباقي

دكتوراه وإستشاري علاج الأورام - جامعة عين شمس
زميل الكلية الملكية لعلاج الأورام - إنجلترا
عضو الجمعية الأوروبية لعلاج الأورام
مستشفى المطرية التعليمي
مستشفى في القاهرة التخصصي
مجموعة مستشفيات كليوباترا

Name: / د/ فاطمة سيد عبد الباقي

Date: 8/9/2026

* 50 yrs, Single, +ve FH د/ فاطمة سيد عبد الباقي
* myomectomy Post menopausal
of fibroid

* 3/2026: post renal obstructive uropathy
• bilat. double-J → ↓↓ S. creat 9

* 4/2026: MRI PA: Rt adnexal & lumbar
heterogenous mass

C.T chest: free

→ Surgical Exploration: Inoperable retroperitoneal
mass → multiple LN biopsy + mass biopsy
+ cytology

البيظة → Reactive lymphadenopathy + fibrocollagenous
tissue fragments + -ve cytology

البيظة → Revision of Pathology: mass → spindle cell
neoplasm schwannoma

⇒ IHC results

→ S100 & Sox10: +ve

CD68: -ve

⇒ low grade spindle cell
neoplasm of neuronal
origin

⇒ U/S guided biopsy: spindle cell
neoplasm for
IHC

منصة جوفتاف (فرم سابقاً)
المستشفى

اسم المريض ثلاثي:
رقم الملف:

تقرير طبي

السيد/

تحية طيبة وبعد،،

إيماء للطلاب المقدم من بشأن تقرير طبي للمريض:
بقسم

نشرف بأن نوضح فيما يلي تقريراً عن حالته :
تاريخ وساعة الدخول

تاريخ وساعة الخروج

وبعد الكشف والفحوصات والمتابعة وجد انه:

١١١ / السيد العيصوي بلقيس حسن
حكيمه سكرتيرة وملازم أول

تحريراً في

الوقت: التاريخ: / /

الطبيب المعالج

يتمتع ،،

اسم المريض ثلاثي:
رقم الملف:

تقرير طبي

السيد /

تحية طيبة وبعد،،،

إيماء للطلب المقدم من

قسم: رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته:

تاريخ وساعة الدخول تاريخ وساعة الخروج:

وبعد الكشف والفحوصات والمتابعة وجد انه:

Stable
NF1

والتي دخلت حيزها في العيادة و
لا توجد

د
C

التاريخ: / /

الوقت:

الطبيب المعالج

يختم

المصدر /

بسمان تقریر طبي للمريض:! ہنسائی سو خوشنمیں! علیہ السلام

تاريخ وساعة الدخول تاريخ وساعة الخروج

مجلس القضاء الاعلى

25 June

2/12/23

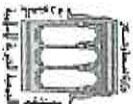
الوقت:

.....

الناصح:

.....

يَعْتَمِدُ



Patient Name : هناء توفيق خطاب
Patient No. : 0330138
Path No. :
Sex : Female
Age : 49 Y
Referring Doc :

Request No. : 142776
Request Date : 09/07/2026
Verification Date : 27/07/2026
Financial Category : نقى - العجوزة
Department : Out Patient

Pathology Report

Path no: 364-26

Clinical data & Nature of specimen:

Right retroperitoneal mass, US guided biopsy.

Gross:

Multiple Fragmented greyish white tissue cores collectively measures 1.8x1.8 cm, totally submitted.

Microscopic:

Examination revealed cores of mass showed alternating hypo and hypercellularity formed of bland looking spindle cells. No evidence of significant atypia, mitosis or necrosis in sections examined.

Diagnosis:

Right retroperitoneal mass, US guided biopsy:

- Spindle cell neoplasm.
- Case is for mandatory immunophenotyping to start with (S100,SMA, Desmin, Ki67).
- Case is clinical and radiological correlation.

Signature

Dr/ Marwa Mahmoud Abd Elkader
FEBP.IFCAP



Patient Name: **هنا توفيق خليل محمد**
MRN: 10110226008792
D.O.B: 17/05/1977
IC No.: 27705170102067

Location: External Clinic
Sex: Female

Surgical Pathology-Diagnosis

Biopsy

Specimen Id	2606131086	Collection Date	13/06/2026 10:29 AM
Accession No	P-26-7012	Received Date	13/06/2026 10:33 AM
Sample Type	TISSUE	Doctor	

CLINICAL HISTORY

A 49 years old female patient presented with inoperable retroperitoneal mass and retroperitoneal lymphadenopathy. Slide revision of incisional biopsy from mass and lymph nodes.

GROSS DESCRIPTION

Three referred paraffin blocks, and Three referred Hx & E stained slides labeled by patient's name in Arabic and coded 1059157, additionally designated 1, 2 and C.B

MICROSCOPIC DESCRIPTION

Examination of the received slides revealed:

- 1- Sections of lymph nodes showing preserved nodal architecture with Reactive Follicular Hyperplasia and sinus histiocytosis. No evidence of malignancy
- 2- Tissue fragment with fibrosis and collagenous stroma, entrapping blood vessels surrounded by lymph plasma cell aggregates. No evidence of malignancy
- 3- The slide received designated (C.B) show fragmented cores formed of spindled cells, fibroblasts and aggregates of large polyhedral cells with granular foamy cytoplasm, ? foamy/lipid laden histiocytes. Occasional hemosiderin laden macrophages are noted with mixed inflammatory cells.

IMMUNOHISTOLOGY

Marker study done revealed positive reaction to S100 and SOX10, while negative for CD68.

DIAGNOSIS

Retroperitoneal mass, Slide revision of core biopsy and mass content: Scanty, Low grade spindle cell neoplasm of neuronal origin. Exact nature and behavior of the tumor cannot be assessed on such scanty biopsy. Correlation with radiologic findings and larger biopsy are advised.

Category

A definitive diagnosis could not be established

Assistant Pathologist:	Registrar:	Signed by:
Dr. Reem Khaled		Dr. Eman Naguib, MD (Assistant Professor)

Reported and electronically endorsed by Dr. Eman Neglab Elhelaly

Dr.

Elia Anis Ishak

M. Sc. Ph. D. Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.

Shady Elia Anis

M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : هناءة توفيق خطاب

Analysis ID : 1059157

Age : 49 Y

Gender : Female

Conf. Code : 5603

Referred By Prof. Dr. : محمد عبد الوالد

Clinical Diagnosis : Inoperable retroperitoneal mass, retroperitoneal lymph node.

Nature of Specimen : Referred blocks and referred slides (retroperitoneal lymph node, mass wall incisional Biopsies and mass content cytology)

Receiving Date : 2026-05-19

Patient ID : 775419

PATHOLOGY REPORT

Gross :

Three blocks and three slides labeled as 26013062.

Microscopic :

Sections prepared from block (1) and examination of slide (1) received revealed multiple lymph nodes with preserved architecture, patent subcapsular sinus with many variably sized reactive follicles. The paracortex showed lymphoplasmacytic infiltrate.

No evidence of specific granuloma.

No evidence of malignancy.

Sections prepared from block (2) and examination of slide (2) received revealed fibrocollagenous tissue with moderate inflammatory reaction.

- Sections prepared from cell block(3) and examination of slide (3) received revealed cores of a tumor formed of bland spindle cells with oval and tapered nuclei in a fibrillary background showing intervening aggregates of xanthoma cells. No malignancy.

Diagnosis :

Inoperable retroperitoneal mass:

1- Retroperitoneal lymph node, Referred block and referred slide (Biopsy), REACTIVE LYMPH NODES, CHRONIC LYMPHADENITIS.

2- Mass wall, Referred block and referred slide (Incisional biopsy), FIBROCOLLAGENOUS TISSUE WITH MODERATE INFLAMMATORY REACTION.

3- Mass content, Referred cell block and referred slide(Cytology), SPINDLE CELL NEOPLASM CONSIDER SCHWANNOMA.

- CASE FOR IMMUNOPHENOTYPING FOR CONFIRMATION.

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak

Shady Elia

Elia Anis

This report is electronically signed , any abrasion or modification would render it void .



عصرة رمسيس - بلك أ - ميدان رمسيس - الدور السادس و السابع

تليفون : ٠١٢١٢٩٤٥١١٥ - ٠١٢٨٧٠٠٥٢١٧



सं. ३०३

Code Number
Patient ID
Referred By Dr.
Branch

91357

الفرع الرئيسي الدقي

Diagnosis: Inoperable Retroperitoneal Mass of the specimen: Retro-peritoneal Lymph Node

Pathology Report

Designated specimens were received as:
 1. Grey reddish and yellowish.

stomach, white and firm tissue pieces collectively measured 2x1.3 cm, totally submitted. Agnament was received in a syringe for cytological examination. Cell block was prepared.

Copic Picture

showed five lymph nodes with reactive follicular hyperplasia. No metastatic malignancy (0/5).
showed fibro collagenous tissue with no epithelial lining and contained focal lymphocytic infiltrate.
showed foamy macrophages mixed with lymphocytes.

sis

Retropertineal Mass, Lymph Node Biopsy, Mass Wall Incisional Biopsy And Mass Content Cytology: Correlate With Clinical And Radiological Findings.

slides enclosed: 3

ed by

Pathologist
Saeed, MD



ي - الحور الأول

[illegible]

1059157
2025

جمهورية مصر العربية
بطاقة تحليل الشخصية



هنا
توفيق خطاب محمد

١٦ ش الجامع - طه قنديل - عزبة حماده
المطرية - القاهرة

٢٧٧.٥١٧.٠١.٢٠.٦٧

LZ0471251



٢٠٢٦/٠٩ ٢٧٧.٥١٧.١٠٢.٦٧

بدون عمل

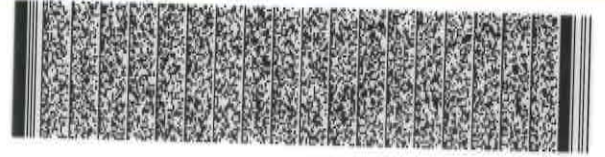
أنثى

مستلمة

أنثى



البطاقة سارية حتى ٢٠٢٣/٠٩/١٦



د/ فاطمة سيد عبد الباقي

دكتوراه وإستشاري علاج الأورام - جامعة عين شمس
زميل الكلية الملكية لعلاج الأورام - إنجلترا
عضو الجمعية الأوروبية لعلاج الأورام
مستشفى المطرية التعليمي
مستشفى في القاهرة التخصصي
مجموعة مستشفيات كليوباترا

Name: / د/ فاطمة سيد عبد الباقي

Date: 8/9/2026

* 50 yrs, Single, +ve FH د/ فاطمة سيد عبد الباقي
* myomectomy Post menopausal
of fibroid

* 3/2026: post renal obstructive uropathy
• bilat. double-J → ↓↓ S. creat 9

* 4/2026: MRI PA: Rt adnexal & lumbar
heterogenous mass

C.T chest: free

→ Surgical Exploration: Inoperable retroperitoneal
mass → multiple LN biopsy + mass biopsy
+ cytology

البيظة → Reactive lymphadenopathy + fibrocollagenous
tissue fragments + -ve cytology

البيظة → Revision of Pathology: mass → spindle cell
neoplasm schwannoma

⇒ IHC results

→ S100 & Sox10: +ve

CD68: -ve

⇒ low grade spindle cell
neoplasm of neuronal
origin

⇒ U/S guided biopsy: spindle cell
neoplasm for
IHC

منصة جوفتاف (فرم سابقاً)
المستشفى

اسم المريض الثلاثي:
رقم الملف:

فريق طلي

15/11/21

تَحِيَّةٌ طَيِّبَةٌ وَبِطَعْنَةٍ

إيماء الطالب المقدم من بشأن تقرير طبي للمريض
.....

رقم طبي بقسم

نفسرف بأن نوضح فيما يلي تقريراً عن حالته :

تاريخ وساعة الدخول تاريخ وساعة الخروج.

وبعد الكشف والفحوصات والمتابعة وجد انه:

1892. 12. 1. 1892. 12. 1.

U.S. 10/1/88

تحريره في

الوقت: التاريخ: / /

الطبيب المعالج

٦٦

اسم المريض ثلاثي:
رقم الملف:

تقرير طبي

السيد /

تحية طيبة وبعد،،،

إيماء الطالب المقدم من

قسم: رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته:

تاريخ وساعة الدخول تاريخ وساعة الخروج:

وبعد الكشف والفحوصات والمتابعة وجد انه:

Stable
NF1

والتي دخلت حيزها في الشهر الأول
والتي كانت

د. م. م. م.

التاريخ: / /

الطبيب المعالج

يختم

المصدر /

بسمان تقرير طبي للمريض: لاجتماعي سوسيفي
.....

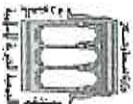
تاريخ وساعة الدخول

myself and my child

251

الوقت: التاريخ: / /

پیچیدگی



Patient Name : هناء توفيق خطاب
Patient No. : 0330138
Path No. :
Sex : Female
Age : 49 Y
Referring Doc :

Request No. : 142776
Request Date : 09/07/2026
Verification Date : 27/07/2026
Financial Category : نقى - العجوزة
Department : Out Patient

Pathology Report

Path no: 364-26

Clinical data & Nature of specimen:

Right retroperitoneal mass, US guided biopsy.

Gross:

Multiple Fragmented greyish white tissue cores collectively measures 1.8x1.8 cm, totally submitted.

Microscopic:

Examination revealed cores of mass showed alternating hypo and hypercellularity formed of bland looking spindle cells. No evidence of significant atypia, mitosis or necrosis in sections examined.

Diagnosis:

Right retroperitoneal mass, US guided biopsy:

- Spindle cell neoplasm.
- Case is for mandatory immunophenotyping to start with (S100,SMA, Desmin, Ki67).
- Case is clinical and radiological correlation.

Signature

Dr/ Marwa Mahmoud Abd Elkader
FEBP.IFCAP



Patient Name: **هانى توفيق عطلي محمد**
MRN: 10110226008792
D.O.B: 17/05/1977
IC No.: 27705170102067

Location: External Clinic
Sex: Female

Surgical Pathology-Diagnosis

Biopsy

Specimen Id 2606131086
Accession No P-26-7012
Sample Type TISSUE
Collection Date 13/06/2026 10:29 AM
Received Date 13/06/2026 10:33 AM
Doctor

CLINICAL HISTORY

A 49 years old female patient presented with inoperable retroperitoneal mass and retroperitoneal lymphadenopathy. Slide revision of incisional biopsy from mass and lymph nodes.

GROSS DESCRIPTION

Three referred paraffin blocks, and Three referred Hx & E stained slides labeled by patient's name in Arabic and coded 1059157, additionally designated 1, 2 and C.B

MICROSCOPIC DESCRIPTION

Examination of the received slides revealed:

- 1- Sections of lymph nodes showing preserved nodal architecture with Reactive Follicular Hyperplasia and sinus histiocytosis. No evidence of malignancy
- 2- Tissue fragment with fibrosis and collagenous stroma, entrapping blood vessels surrounded by lymph plasma cell aggregates. No evidence of malignancy
- 3- The slide received designated (C.B) show fragmented cores formed of spindled cells, fibroblasts and aggregates of large polyhedral cells with granular foamy cytoplasm, ? foamy/lipid laden histiocytes. Occasional hemosiderin laden macrophages are noted with mixed inflammatory cells.

IMMUNOHISTOLOGY

Marker study done revealed positive reaction to S100 and SOX10, while negative for CD68.

DIAGNOSIS

Retroperitoneal mass, Slide revision of core biopsy and mass content: Scanty, Low grade spindle cell neoplasm of neuronal origin. Exact nature and behavior of the tumor cannot be assessed on such scanty biopsy. Correlation with radiologic findings and larger biopsy are advised.

Category

A definitive diagnosis could not be established

Assistant Pathologist:
Dr. Reem Khaled

Registrar:

Signed by:
Dr. Eman Naguib, MD (Assistant Professor)

Reported and electronically endorsed by Dr. Eman Naguib Elhelaly

Dr.

Elia Anis Ishak

M. Sc. Ph. D. Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.

Shady Elia Anis

M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : هناءة توفيق خطاب

Analysis ID : 1059157

Age : 49 Y

Gender : Female

Conf. Code : 5603

Referred By Prof. Dr. : محمد عبد الوالد

Clinical Diagnosis : Inoperable retroperitoneal mass, retroperitoneal lymph node.

Nature of Specimen : Referred blocks and referred slides (retroperitoneal lymph node, mass wall incisional Biopsies and mass content cytology)

Receiving Date : 2026-05-19

Patient ID : 775419

PATHOLOGY REPORT

Gross :

Three blocks and three slides labeled as 26013062.

Microscopic :

Sections prepared from block (1) and examination of slide (1) received revealed multiple lymph nodes with preserved architecture, patent subcapsular sinus with many variably sized reactive follicles. The paracortex showed lymphoplasmacytic infiltrate.

No evidence of specific granuloma.

No evidence of malignancy.

Sections prepared from block (2) and examination of slide (2) received revealed fibrocollagenous tissue with moderate inflammatory reaction.

- Sections prepared from cell block(3) and examination of slide (3) received revealed cores of a tumor formed of bland spindle cells with oval and tapered nuclei in a fibrillary background showing intervening aggregates of xanthoma cells. No malignancy.

Diagnosis :

Inoperable retroperitoneal mass:

1- Retroperitoneal lymph node, Referred block and referred slide (Biopsy), REACTIVE LYMPH NODES, CHRONIC LYMPHADENITIS.

2- Mass wall, Referred block and referred slide (Incisional biopsy), FIBROCOLLAGENOUS TISSUE WITH MODERATE INFLAMMATORY REACTION.

3- Mass content, Referred cell block and referred slide(Cytology), SPINDLE CELL NEOPLASM CONSIDER SCHWANNOMA.

- CASE FOR IMMUNOPHENOTYPING FOR CONFIRMATION.

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak

Shady Elia

Elia Anis

This report is electronically signed , any abrasion or modification would render it void .



عصرة رمسيس - بلك أ - ميدان رمسيس - الدور السادس و السابع

تليفون : ٠١٢١٢٩٤٥١١٥ - ٠١٢٨٧٠٠٥٢١٧



सं. ३०३

Code Number
Patient ID
Referred By Dr.
Branch

91357

الفرع الرئيسي الدقي

Diagnosis: Inoperable Retroperitoneal Mass
if the specimen: Retro-peritoneal Lymph No

Pathology Report

Designated specimens were received as:
le grey reddish and yellowish.

was received in a syringe for cytological examination. Cell block was prepared.

Copic Picture

showed five lymph nodes with reactive follicular hyperplasia. No metastatic malignancy (0/5). Lymph nodes showed fibro collagenous tissue with no epithelial lining and contained focal lymphoid follicles. Lymph nodes showed foamy macrophages mixed with lymphocytes.

...multicentric giant cells.

High Nodes With Histomorphological Features of Retroperitoneal Mass, Lymph Node Biopsy, Mass Wall Incisional Biopsy, and Hemotyping.

Check With Features Of Xantho-granulomatous Inflammatory Process.
For Cytologically Malignant Cells.
Correlate With Clinical And Radiological Findings.

slides enclosed: 3

Pathologist
Saeed, MD



ي - الحور الأول

حقوقي: - شارع التحرير أمام مقر البحوث - الصفحة ٧٣. ص ٥٤٣. ص ٢١١.



1059157
2025